

AI Backend

Choose AI model:

- ☐ Offline Demo
- ☒ Gemini (free)

Gemini API Key

[Redacted Gemini API Key]

Key set

Policy Documents

Document source:

- ☐ Sample CMS Policies
- ☒ Upload your own

Supports .txt and .pdf files



# Healthcare Policy Content Analyzer

AI-powered policy comparison, rule extraction & provider alert generation  
t -- Likitha KB

Analysis completed in 189.6 seconds

| Key Changes ... | HIGH Risks    | MEDIUM Risks | Denial-Risk R... | Documents C... |
|-----------------|---------------|--------------|------------------|----------------|
| 31              | 6             | 1            | 34/37            | 2 P...         |
|                 | ↑ action r... | ↑ monitor    |                  |                |

Comparing: 1.pdf vs 2.pdf

Policy Change Summary    Extracted Coding Rules    Provider Alert    **Policy Diff Viewer**

# Side-by-Side Document Comparison

Lines prefixed with + are new or changed in the updated document.

Removed / Changed in Old Document

Added / Changed in New Document

16 paragraph(s) removed or replaced

49 paragraph(s) added or replaced

- 1995 DOCUMENTATION GUIDELINES  
FOR EVALUATION AND MANAGEMENT  
SERVICES  
I. INTRODUCTION  
  
WHAT IS DOCUMENTATION AND WHY  
IS IT IMPORTANT?  
  
Medical record documentation is  
required to record pertinent facts,  
findings, and observations about an  
individual's health history including  
past and present illnesses,  
examinations, tests, treatments, and  
outcomes. The medical record  
chronologically doc...

- the medical necessity and

+ 1997 DOCUMENTATION GUIDELINES  
FOR EVALUATION AND MANAGEMENT  
SERVICES  
TABLE OF CONTENTS  
  
Introduction  
.....  
..... 2  
What Is Documentation and Why Is it  
Important?..... 2  
What Do Payers Want and Why?  
.....  
... 2  
  
General ...

+ Documentation of the Complexity of  
Medical Decision Making .....

appropriateness of the diagnostic and/or therapeutic services provided; and/or that services provided have been accurately reported.

II. GENERAL PRINCIPLES OF MEDICAL RECORD DOCUMENTATION

The principles of documentation listed below are applicable to all types of medical and surgical services in all settings. For Evaluation and Management (E/M) services, the ...

- II. DOCUMENTATION OF E/M SERVICES

This publication provides definitions and documentation guidelines for the three key components of E/M services and for visits which consist predominately of counseling or coordination of care. The three key components--history, examination, and medical decision making--appear in the descriptors for office and other outpatient services, hospital observation...

43 Number of Diagnoses or Management Options  
..... 44 Amount and/or Complexity of Data to Be Reviewed ..... 45 Risk of Significant Complications, Morbidity, and/or Mortality ..... 46 Table of Risk .....

+ I. INTRODUCTION  
WHAT IS DOCUMENTATION AND WHY IS IT IMPORTANT?

Medical record documentation is required to record pertinent facts, findings, and observations about an individual's health history including past and present illnesses, examinations, tests, treatments, and outcomes. The medical record chronologically documents the care of the patient and is an important element contributing ...

+ II. GENERAL PRINCIPLES OF MEDICAL RECORD DOCUMENTATION  
The principles of documentation listed

#### Documentation

– As an example, newborn records may include under history of the present illness (HPI) the details of mother's pregnancy and the infant's status at birth; social history will focus on family structure; family history will focus on congenital anomalies and hereditary disorders in the family. In addition, information on growth and development and/or nutrition will be recorded. Although not spec...

– DG: The CC, ROS and PFSH may be listed as separate elements of history, or they may be included in the description of the history of the present illness.  
DG: A ROS and/or a PFSH obtained during an earlier encounter does not need to be re-recorded if there is evidence that the physician reviewed and updated the previous information. This may occur when a physician updates his/her own record...

below are applicable to all types of medical and surgical services in all settings. For Evaluation and Management (E/M) services, the nature and amount of physician work and documentation varies by type of service, place of service and the patient's status. The general principles listed below may b...

#### + III. DOCUMENTATION OF E/M SERVICES

This publication provides definitions and documentation guidelines for the three key components of E/M services and for visits which consist predominately of counseling or coordination of care. The three key components--history, examination, and medical decision making--appear in the descriptors for office and other outpatient services, hospital observatio...

+ children, adolescents and pregnant women may have additional or

### – HISTORY OF PRESENT ILLNESS (HPI)

The HPI is a chronological description of the development of the patient's present illness from the first sign and/or symptom or from the previous encounter to the present. It includes the following elements:

location;

quality;

severity;

duration;

timing;

context;

modifying factors; and

associated signs and symptoms.

Brief an...

women may have additional or

modified information recorded in each history and examination area.

As an example, newborn records may include under history of the present illness (HPI) the details of mother's pregnancy and the infant's status at birth; social history will focus on family structure; family history will focus on congenital anomalies and hered...

+ DG: The CC, ROS and PFSH may be listed as separate elements of history, or they may be included in the description of the history of the present illness.

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– For purposes of ROS, the following systems are recognized:

+ HISTORY OF PRESENT ILLNESS (HPI)  
The HPI is a chronological description

|                                                    |
|----------------------------------------------------|
| Constitutional symptoms (e.g., fever, weight loss) |
| Eyes                                               |
| Ears, Nose, Mouth, Throat                          |
| Cardiovascular                                     |
| Respiratory                                        |
| Gastrointestinal                                   |
| Genitourinary                                      |
| Musculoskeletal                                    |
| Integumentary (skin and/or breast)                 |
| Neurological                                       |
| Psychiatric                                        |
| Endocrine                                          |
| Hematologic/Lymphatic                              |
| Allergic/Immunologic                               |
| A problem per...                                   |

of the development of the patient’s present illness from the first sign and/or symptom or from the previous encounter to the present. It includes the following elements:  
location , quality , severity, duration, timing, context , modifying factors, and associated signs and symptoms.

Brief and extended...

+ REVIEW OF SYSTEMS (ROS)

A ROS is an inventory of body systems obtained through a series of questions seeking to identify signs and/or symptoms that the patient may be experiencing or has experienced.

For purposes of ROS, the following systems are recognized: Constitutional Symptoms (eg, fever, weight loss) Eyes Ears, Nose, Mouth, and Throat Cardiovascular Respiratory Gastroin...

+ PAST FAMILY AND/OR SOCIAL

– A complete ROS inquires about the system(s) directly related to the problem(s) identified in the HPI plus all additional body systems.

DG: At least ten organ systems must be reviewed. Those systems with positive or pertinent negative responses must be individually documented. For the remaining systems, a notation indicating all other systems are negative is permissible. In the absence of s...

– DG: At least one specific item from each of the three history areas must be documented for a complete PFSH for the following categories of E/M services: office or other outpatient services, new patient; hospital observation services; hospital inpatient services, initial care; consultations; comprehensive nursing facility assessments; domiciliary care, new patient; and homecare, new patient. ...

## HISTORY (PFSH)

The PFSH consists of a review of three areas:

past history (the patient's past experiences with illnesses, operations, injuries and treatments);

family history (a review of medical events in the patient's family, including diseases which may be hereditary or place the patient at risk); and

social history (an age appropriate review of past ...

+ DG: At least one specific item from each of the three history areas must be documented for a complete PFSH for the following categories of E/M services: office or other outpatient services, new patient; hospital observation services; hospital inpatient services, initial care; consultations; comprehensive nursing facility assessments; domiciliary care, new patient; and homecare, new patient. ...

– For purposes of examination, the following organ systems are recognized:

Constitutional (e.g., vital signs, general appearance)

Eyes

Ears, nose, mouth, and throat

Cardiovascular

Respiratory

Gastrointestinal

Genitourinary

Musculoskeletal

Skin

Neurologic

Psychiatric

Hematologic/lymphatic/immunologic

The extent of examinations performed and documente...

new patient; home care, new patient.  
B...

+ A general multi-system examination or a single organ system examination may be performed by any physician, regardless of specialty. The type (general multi- system or single organ system) and content of examination are selected by the examining physician and are based upon clinical judgment, the patient's history, and the nature of the presenting problem(s).  
The content and documentation req...

+ Problem Focused Examination – should include performance and documentation of one to five elements identified by a bullet (•) in one or more organ system(s) or body area(s).  
Expanded Problem Focused Examination – should include performance and documentation of at least six elements identified by a bullet (•) in one or more organ system(s) or

### – C. DOCUMENTATION OF THE COMPLEXITY OF MEDICAL DECISION MAKING

The levels of E/M services recognize four types of medical decision making (straight- forward, low complexity, moderate complexity, and high complexity). Medical decision making refers to the complexity of establishing a diagnosis and/or selecting a management option as measured by:

the number of possible diagnoses and/or the n...

– Generally, decision making with respect to a diagnosed problem is easier than that for an identified but undiagnosed problem. The number and type of diagnostic tests employed may be an indicator of the number of possible diagnoses. Problems which are improving or resolving are less complex than those which are worsening or failing to change as expected. The need to seek advice from others is a...

body area(s). Detailed Examination – should incl...

+ Comprehensive Examination – should include performance of all elements identified by a bullet (•), whether in a shaded or unshaded box. Documentation of every element in each box with a shaded border and at least one element in a box with an unshaded border is expected.

### CONTENT AND DOCUMENTATION REQUIREMENTS

#### General Multi-System Examination

System/Body Area  
Elements of Examinatio...

+ System/Body Area Elements of Examination  
Respiratory Assessment of respiratory effort (eg, intercostal retractions, use of accessory muscles, diaphragmatic movement)  
Percussion of chest (eg, dullness, flatness, hyperresonance)

Palpation of chest (eg, tactile fremitus)

– occasion the physician who ordered a test may personally review the image, tracing or specimen to supplement information from the physician who prepared the test report or interpretation; this is another indication of the complexity of data being reviewed.

DG: If a diagnostic service (test or procedure) is ordered, planned, scheduled, or performed at the time of the E/M encounter, the typ...

– RISK OF SIGNIFICANT COMPLICATIONS, MORBIDITY, AND/OR MORTALITY

The risk of significant complications, morbidity, and/or mortality is based on the risks associated with the presenting problem(s), the diagnostic procedure(s), and the possible management options.

DG: Comorbidities/underlying diseases or other factors that increase the complexity of medical decision making

Auscultation of lungs (eg, breath sounds, adventitious sounds, rubs)  
Cardiovascular  
Palpation...

+ System/Body Elements of Examination Area Genitourinary MALE:  
Examination of the scrotal contents (eg, hydrocele, spermatocele, tenderness of cord, testicular mass)

Examination of the penis

Digital rectal examination of prostate gland (eg, size, symmetry, nodularity, tenderness)

FEMALE:

Pelvic examination (with or without specimen collection for s...

+ System/Body Elements of Examination Area Musculoskeletal  
Examination of gait and station  
Inspection and/or palpation of digits

by increasing the ...

- Table of Risk
- Level of Risk
- Presenting Problem(s) Diagnostic
- Procedure(s) Ordered Management
- Options Selected
- Minimal
- One self-limited or minor problem, eg, cold, insect bite, tinea corporis
- Laboratory tests requiring venipuncture
- Chest x-rays
- EKG/EEG
- Urinalysis Ultrasound, eg, echocardiography
- KOH prep
- Rest Gargles
- Elastic bandages Superficial dressings
- Low ...

- D. DOCUMENTATION OF AN ENCOUNTER DOMINATED BY COUNSELING OR COORDINATION OF

and nails (eg, clubbing, cyanosis, inflammatory conditions, petechiae, ischemia, infections, nodes)  
Examination of joints, bones and muscles of one or more of the following six areas: 1) head and neck; 2) spine, ribs and pelvis; 3) right upp...

- + Content and Documentation
- Requirements Level of Exam Perform and Document: Problem Focused One to five elements identified by a bullet. Expanded Problem Focused At least six elements identified by a bullet. Detailed At least two elements identified by a bullet from each of six areas/systems OR at least twelve elements identified by a bullet in two or more areas/systems. Co...

+ Cardiovascular Examination  
System/Body Area  
Elements of Examination  
Constitutional

CARE

In the case where counseling and/or coordination of care dominates (more than 50%) of the physician/patient and/or family encounter (face-to-face time in the office or other outpatient setting or floor/unit time in the hospital or nursing facility), time is considered the key or controlling factor to qualify fo...

Constitutional

Measurement of any three of the following seven vital signs: 1) sitting or standing blood pressure, 2) supine blood pressure, 3) pulse rate and regularity, 4) respiration, 5) temperature, 6) height, 7) weight (May be measured and recorded by ancillary staff)  
General appearance of ...

+ System/Body Area  
Elements of Examination  
Chest (Breasts)

Gastrointestinal (Abdomen)

Examination of abdomen with notation of presence of masses or tenderness Examination of liver and spleen Obtain stool sample for occult blood from patients who are being considered for thrombolytic or anticoagulant therapy Genitourinary (Abdomen)  
Lymphatic

Musculoskeleta...

[29 more added paragraphs — full analysis  
ran on 100% of the document]

Diff summary: 49 paragraph(s) added, 16 removed, ~0 unchanged | 16 → 49 total  
paragraphs